

PW233: Care Across the Lifespan

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Course Learning Outcomes (CLO #1)

- recall the social determinants of health and their impact on clients and their family
- recall holistic health
- recall humility, sensitivity, awareness, and competence related to cultural care
- recall the impacts of colonization and intergenerational trauma on Indigenous people
- recall the impact of culture and spiritual beliefs on the client and their family including unique, vulnerable, and marginalized groups
- identify culturally sensitive approaches when caring for clients and their family including unique, vulnerable, and marginalized groups throughout the lifespan
- identify unique local, provincial, and national cultural supports for Indigenous people across the lifespan (i.e., Jordan's Principle, Thunder Bay Indigenous Friendship Centre, community programs, Dilico, Tikinagan)
- examine traditional, non-traditional, and alternative healing practices for clients and their family

REVIEW Dimensions of Health

- **Health** is more than just not being sick
- It looks at the **whole person**
- All areas of life are connected and affect each other
- Each dimension can improve or decline
- Balance between them = better well-being and resilience

[\(Government of Canada, 2024\)](#)



Health Promotion

- Health is shaped by more than just genetics and lifestyle
- Where people are born, live, work, and age matters

(Government of Canada, 2024)

- **Health Promotion:** helping people take control of and improve their health
 - Not just about individuals
 - Also about changing social, economic, and environmental conditions
- ***Health is shaped by both choices and environment***

Health Promotion

- **Social Determinants of Health**
 - Factors that influence health, extending beyond personal biology and genetics
 - Significant contributors to health inequities



Social Determinants of Health (Key Factors)

1. Income and social status
2. Employment and working conditions
3. Education and literacy
4. Childhood experiences
5. Physical environments
6. Social supports and coping skills
7. Healthy behaviours
8. Access to health services
9. Biology and genetic endowment
10. Gender
11. Culture
12. Race / Racism

(Government of Canada, 2024)

Social Determinants of Health

- Focus on social & economic conditions (e.g., income, education, employment)
- Influence a person's position in society
- Discrimination, racism, and historical trauma impact health outcomes
- Particularly important for groups such as Indigenous Peoples, LGBTQ+, and Black Canadians

[\(Government of Canada, 2024\)](#)

Health Inequalities and Health Inequity

Health inequalities are differences in health status between different groups.

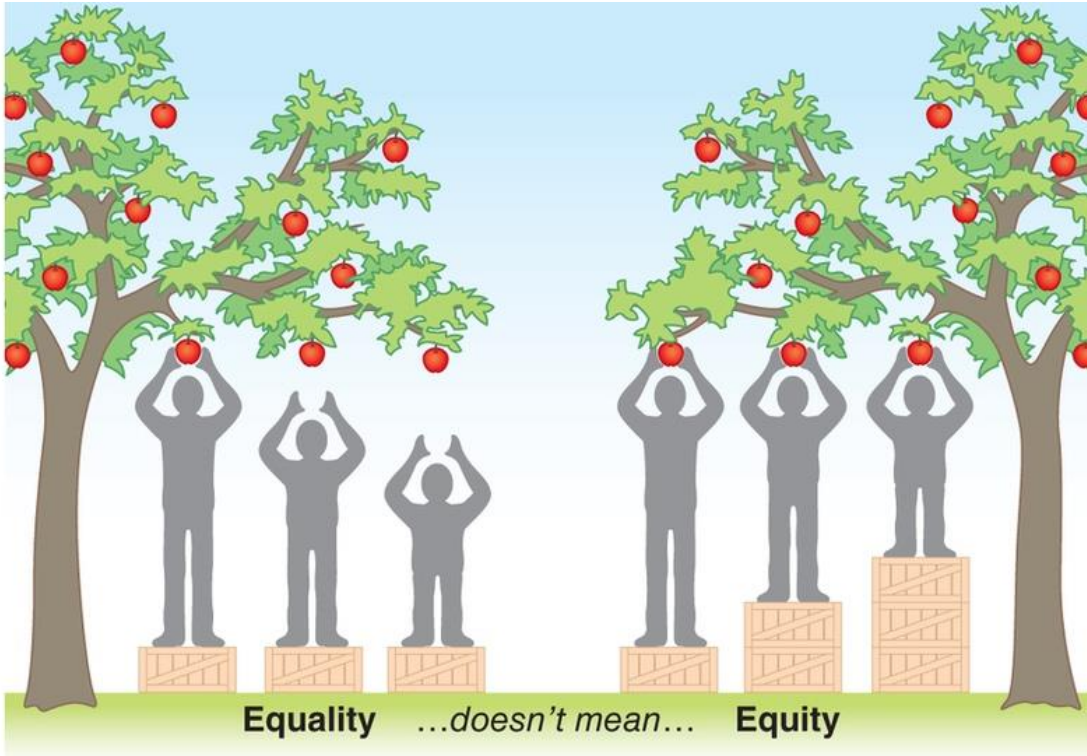
- Can be due to:
 - Genetics
 - Personal choices (e.g., exercise, alcohol use)
 - Social factors (e.g., income, education)

Health inequity: “Unfair and avoidable health differences between population groups arising from social, economic, demographic and geographic conditions” (NCCDH, 2023, p. 3)

Health Inequality OR Inequity?

1. Two people develop the same illness, but one recovers faster due to stronger genetics.
2. A low-income community has limited access to fresh food and higher rates of diabetes.
3. Older adults experience more chronic illness than younger adults.
4. Indigenous communities have less access to healthcare services compared to urban populations.
5. A person chooses to smoke and develops lung disease.

Health Equity



- **Health equity** is the accomplishment of the highest level of health for all people.
 - Goal: Elimination of health inequalities

Main Actions to Improve Health Equity:

1. Improve daily living conditions
2. Address unfair systems
3. Increase awareness & measure the problem

Strong communities = better health outcomes

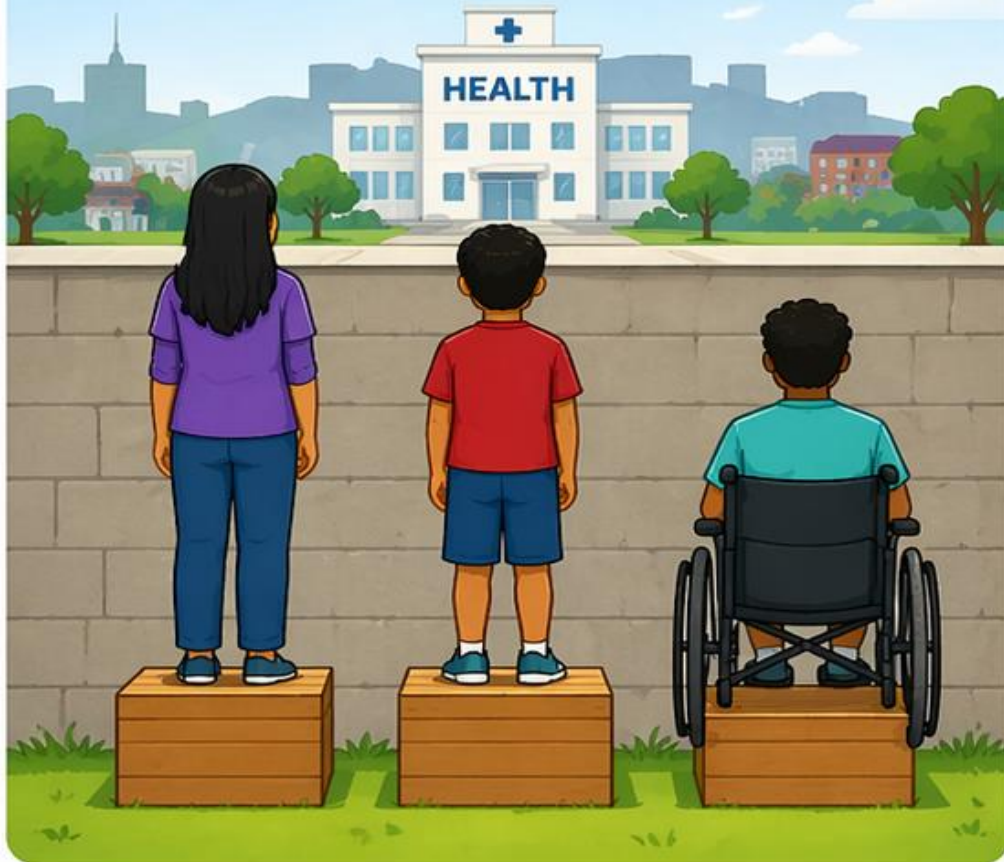
Equality is *fair treatment*, but equity is *fair outcomes*.

HEALTH EQUITY

Everyone has a fair and just opportunity
to be as healthy as they can be

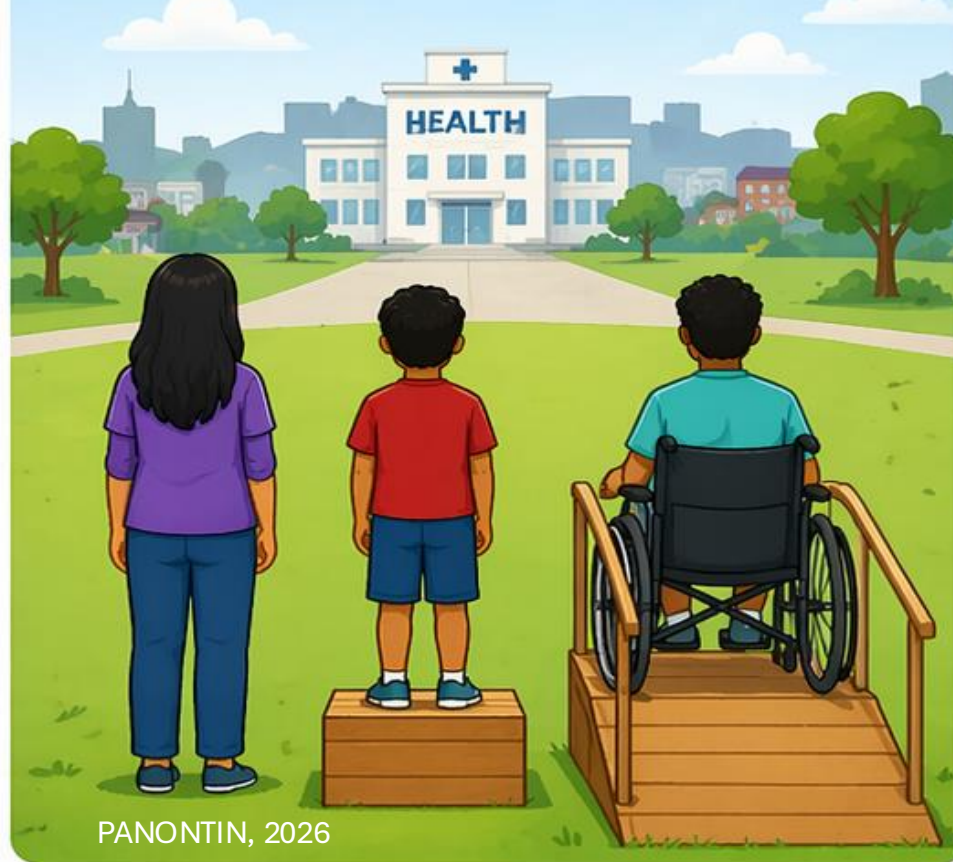
EQUALITY

Everyone gets the same support.
Barriers remain.



EQUITY

Everyone gets the support they need.
Barriers are removed.



PANONTIN, 2026

WHAT CREATES HEALTH EQUITY?



Quality education



Good jobs
and fair income



Safe and affordable
housing



Safe neighborhoods
and transportation



Access to health care
and services



Inclusion, respect
and opportunity
for all

Role of the PSW

- Health care providers have a responsibility to address inequities

They can:

- ✓ Ask patients about social challenges (income, housing, etc.)
- ✓ Help connect them to supports and services
- ✓ Provide culturally safe care
- ✓ Work with communities and public health to advocate for change

Walk in their Shoes

Guiding Questions

1. What health challenges might this client face?
2. What barriers impact their health?
3. How might this affect their family?
4. Is this a **health inequality or inequity**? Why?
5. What could improve their situation?

CLIENT A

Single Parent, Low Income,
Remote Northern Community



- Single parent
- Low income
- Lives in a remote Northern community
- Limited access to fresh food and healthcare

CLIENT B

Middle-Income Family
in an Urban Area



- Middle-income family
- Stable jobs
- Lives in an urban area
- Access to healthcare, transportation and services

CLIENT C

Elderly Indigenous Client
Living Alone



- Elderly Indigenous client
- Lives alone
- History of residential school trauma
- Limited trust in the healthcare system

Population Mosaic of Canada

- **Diversity**
 - Ethnocultural, religious, linguistic, gender identity, abilities, disabilities
- **Multiculturalism**
 - Ideas and ideals related to respect for, and celebration of, our cultural diversity, operationalized at the federal, provincial, and municipal levels
- **Cultural Pluralism**
 - Diverse groups maintaining their unique cultural identities while living together harmoniously



Population Statistics and Demographic Trends

- Canada's population—over 40 million in 2023
- Population growth—natural increase
- International migration has had a great influence
- Number of temporary (nonpermanent residents) greater than the number of permanent immigrants
- Just over half of the population is female
- Aging—driven by increased life expectancy, lower birth rates, and baby boomers

Immigration in Canada

- Europeans began migrating to Canada in the sixteenth century.
 - Brought infectious diseases that Indigenous peoples had no immunity to
 - Displaced Indigenous people from land
- Since the 1960s, immigrants have come from Asia, the Caribbean, and Africa.
- Immigrants arriving today are healthy; however, over time their health status decreases.



Ethnicity, Ethnic and Minority Groups

- **Ethnicity** is a complex and evolving concept that is not synonymous with either race or culture.
- A ***minority group*** refers to marginalized status related to ethnicity, religion, gender, or sexual orientation.
- Ethnic groups have their unique beliefs and attitudes about health and health care services.

Culture, Values, and Value Orientation

- **Culture**
 - Common values, beliefs, traditions, and behaviours of a group of people that differ from another group
- **Values**
 - Belief about the worth of something
 - Standards that influence behaviour and thinking
- **Value orientation**
 - Values learned and shared through socialization
 - Reflect “personality type” of a particular society

Culture and Health

- Culture can affect how health and illness are perceived.
- It can influence:
 - What they think causes illness
 - How they feel about symptoms
 - When/where they seek care
 - What treatments they trust/prefer

Cultural Competence and Humility

- **Cultural competence**
 - Ability to apply knowledge and skills appropriately in interactions with individuals of different cultures and achieve equity in health quality and outcomes
- **Cultural humility**
 - A lifelong process of self-reflection and critique (role of learner) that includes the recognition of power imbalances when working in partnership with patients

Culture Humility: Application

- PSWs need to have self-awareness of their own biases and assumptions.
 - Recognize differences in culture to provide culturally humble care.
 - Be open to new ideas.
 - Challenge power dynamics.
 - Provide meaningful interactions.

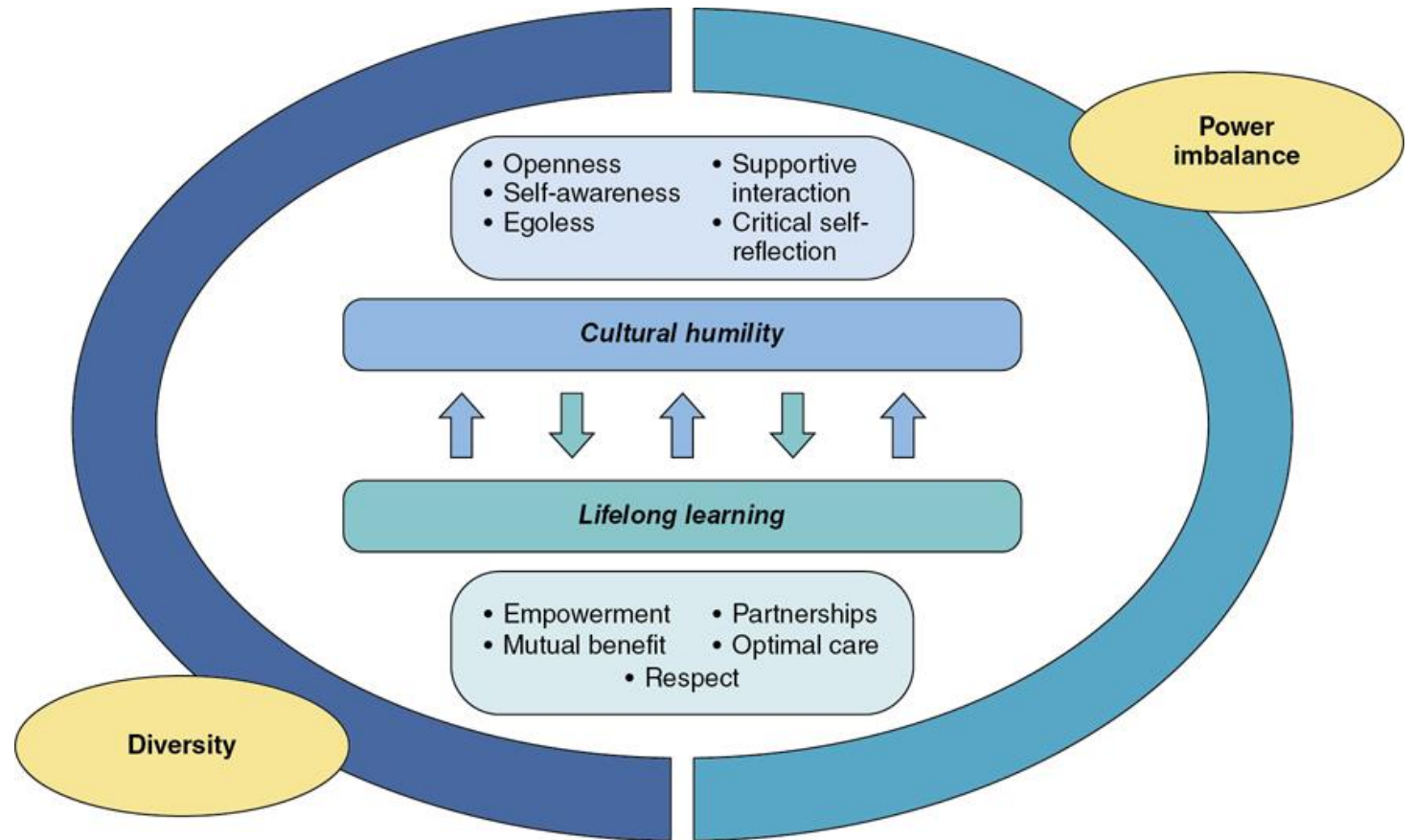


FIG. 3.1 Cultural humility is an approach that can be used to help recognize diversity and power imbalances through personal reflection, self-awareness, relational openness, and social justice. Source: (From Foronda, C., Baptiste, D. L., Reinholdt, M. M., et al. [2016]. Cultural humility: A concept analysis. *Journal of Transcultural Nursing*; 27[3], 210–217. <https://doi.org/10.1177/1043659615592677>)

Trauma-Informed Care

- Trauma is not always visible.
- Trauma-informed care (TIC)
 - Patient-centred approach
 - Attuned to unique experiences of trauma survivors
- PSWs should always ask:
 - Does this make the person feel safe?
 - Am I showing respect?
 - Am I building trust?

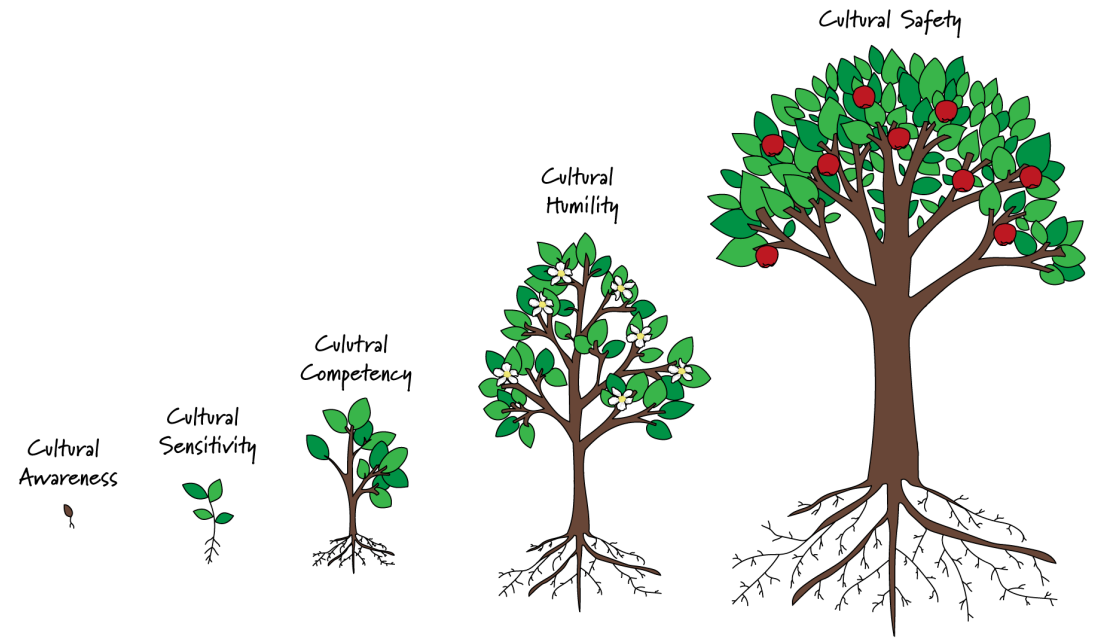


Principles of Trauma- Informed Care



Cultural Safety

- **Cultural safety** goes beyond awareness and acknowledgement of differences to understanding the limitations of cultural competency.
- An outcome of respectful engagement that results in people feeling safe when receiving health care
- Is grounded in the decolonization of health care



Traditional Healing vs Western Medicine

- Health and healing are influenced by culture
- Two main approaches:
 - Traditional healing systems
 - Western (biomedical) medicine
- Many people use both systems together



Traditional Healing vs Western Medicine

- **Traditional Healing:**
 - Based on beliefs, values, traditions
 - Focus on holistic care (mind, body, spirit)
 - Passed down through generations
- **Western Medicine:**
 - Based on science and evidence
 - Focus on diagnosis and treatment
 - Used in hospitals and clinics
- Both systems are shaped by cultural beliefs

Why People Chose Traditional Healing



- Familiar and trusted healer
- Same language and culture
- Accessible and affordable
- More comfortable environment
- Some people:
 - Use traditional healing first
 - Turn to Western care if needed
 - Use both together

Role of Support Workers in Culturally Safe Care

- Ask about traditional medicine use
 - Assess for interactions with treatments
 - Provide respectful, non-judgmental care
 - Adapt care to patient beliefs and needs
-  **Avoid:**
- Ethnocentrism (thinking your way is “better”)
 - Stereotyping (assuming all people are the same)

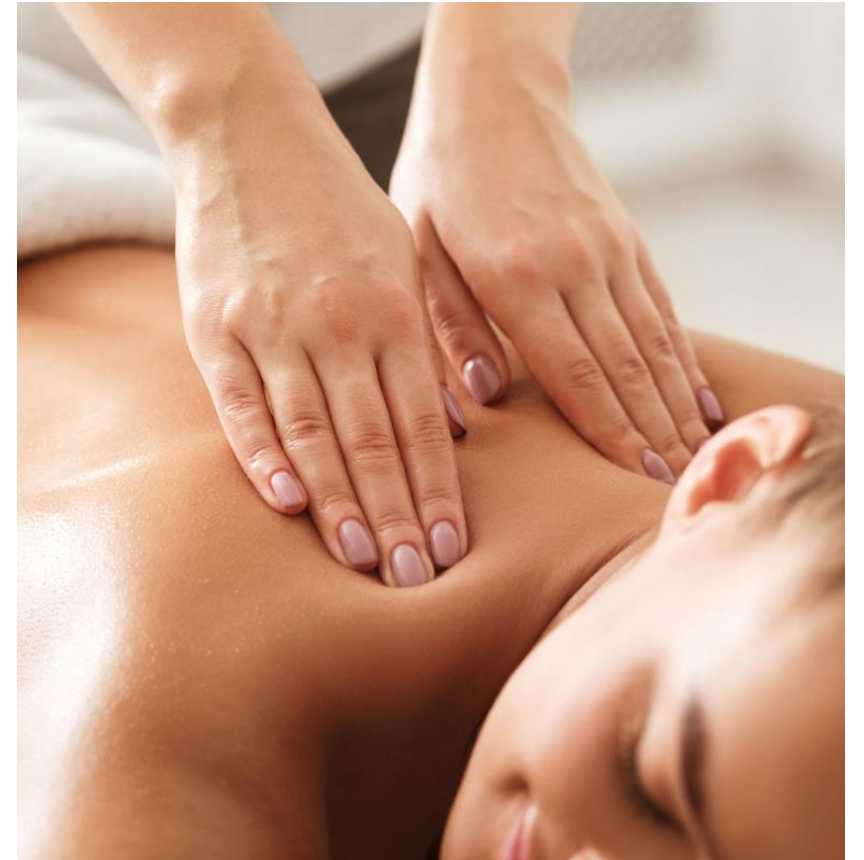
Traditional & Complementary Medicine (T&CM)

Also called:

- Complementary Medicine (CM)
- Complementary & Alternative Medicine (CAM)

Examples:

- Acupuncture
- Massage therapy
- Naturopathy
- Indigenous healing practices
- World Health Organization supports safe use
- Use is increasing worldwide



Integrative & Holistic Care

- Growing trend in Canada: Integrative care
 - Combines traditional + Western medicine
- Focus on:
 - Patient choice
 - Whole-person care
 - Wellness, not just illness
- Support Workers need knowledge of:
 - Benefits
 - Risks
 - Safe use of different therapies

Role of the Support Worker

- Culture shapes health beliefs and behaviours
- Traditional healing remains important worldwide
- Patients may use multiple approaches
- Support Workers must:
 - Be culturally aware
 - Assess safely
 - Provide patient-centered care

Marginalized and Vulnerable Groups

Marginalization: being excluded or disadvantaged by society in ways that make it harder to fully participate or succeed.

*Note: marginalization is not the same for every individual. It depends on context, resources, and intersecting identities



Marginalized and Vulnerable Groups



Think about why each group (as a whole) may be seen as marginalized...

- Indigenous people
- Immigrants/refugees
- Homeless populations
- Gender- and Sexual- diverse populations
- Diverse ability populations

Indigenous People

- Three distinct groups of Indigenous people with unique histories and culture in Canada
 - First Nations
 - Inuit
 - Métis
- *Indian Act*—1876: Was created to assimilate First Nations people (Indians) into dominant Canadian culture and to control their lands
- The *Indian Act* specified a person's eligibility for Indian status.
- Status Indians or Registered Indians have certain rights and benefits compared to Inuit and Métis.



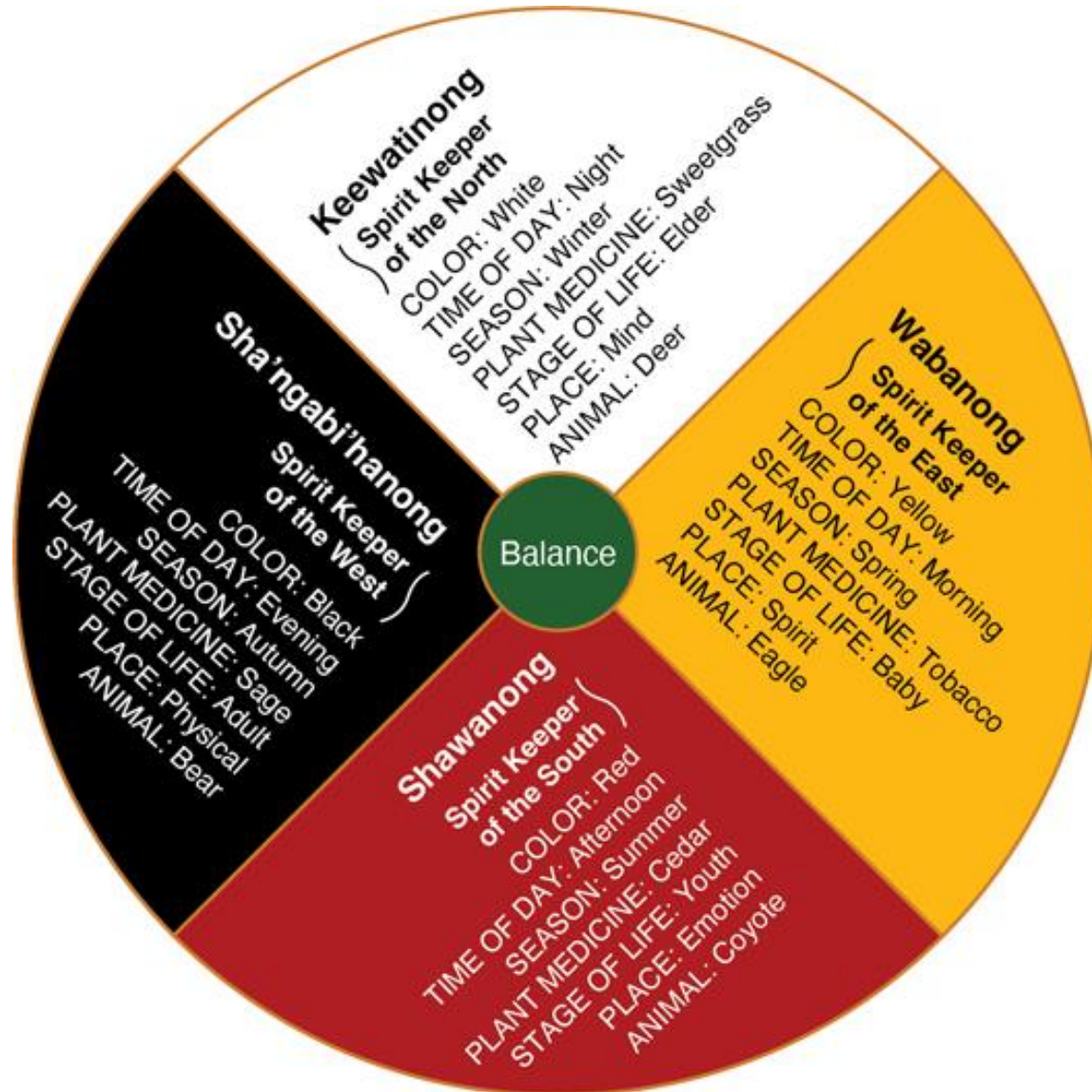
The Health Status of Indigenous Peoples

- View health holistically.
 - Includes emotional, mental, and spiritual well-being
- In the past, documented health status focused on the low standards of health experienced, which are directly related to **colonization**.
 - While important to note deficits to prioritize care, these accounts assume that Indigenous people are ill and experience more disparities and deficits.
 - Impacted by Indigenous determinants of health

Indigenous Knowledges (IK)

- Connections to land, water, and sky
- Intergenerational knowledge sharing—passing down knowledge through communication systems
- Dynamic and transformational
- Nurses need to be aware of IK to provide culturally safe, relational nursing practice.
 - Foster an environment of respect and acknowledgement.

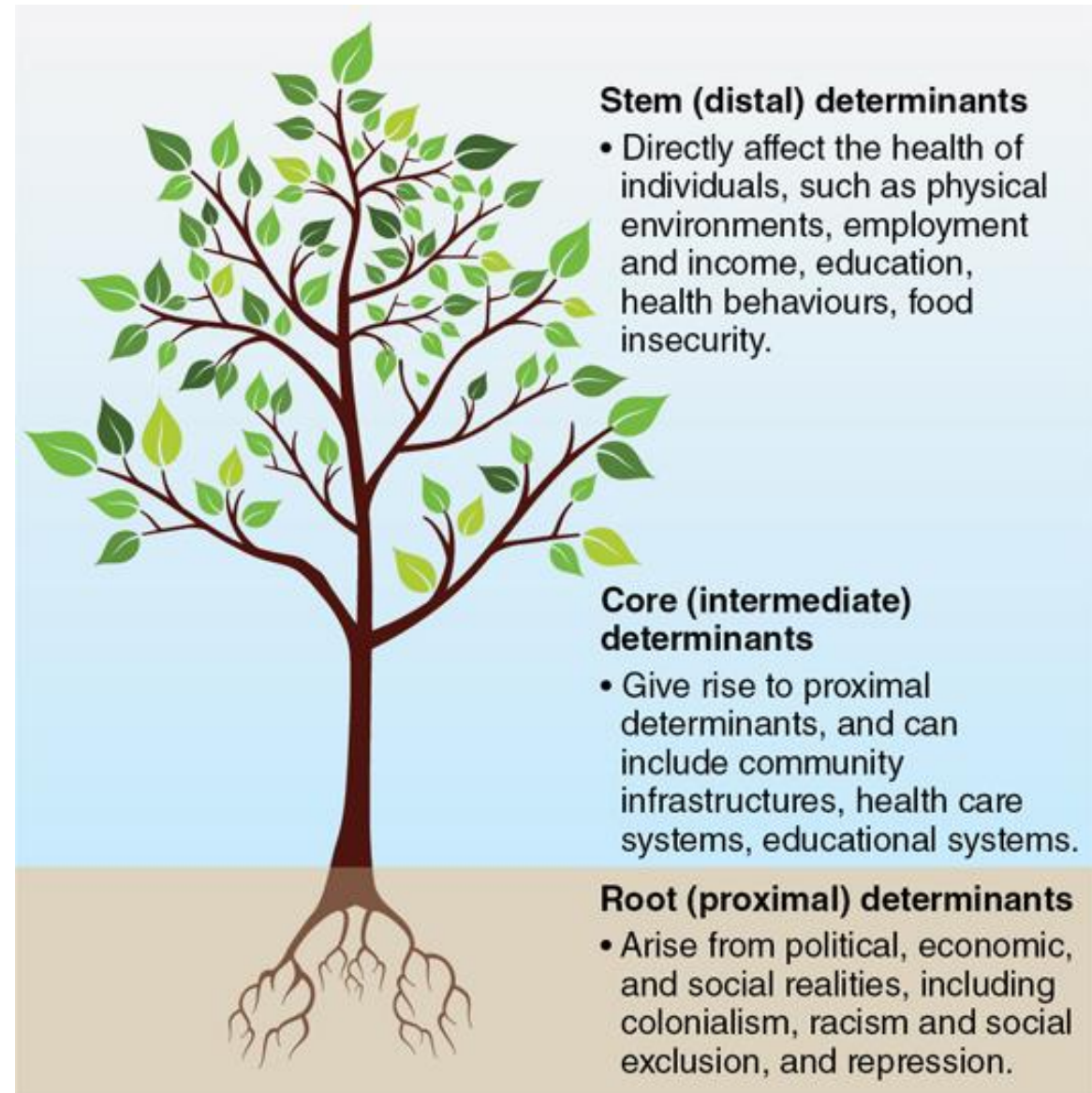
The Health Status of Indigenous Peoples



- Health promotion should focus on what is strong, not what is wrong.
 - Diverse understanding of health and well-being
 - Drawing on community knowledge
- Focus on strengths of Indigenous people and intergenerational knowledge sharing

Indigenous Determinants of Health

FIG. 4.1 Social determinants of Indigenous peoples' health.



Health Issues of Indigenous Populations

- Directly linked to **colonialism** through the forced residential school system, the Sixties Scoop.
- Current social and economic living conditions predispose people to illnesses.
- Truth and Reconciliation Commission (TRC) of Canada documented the assimilation of Indigenous peoples.
- The TRC outlines 94 calls for action, including reducing health inequalities.



Photo credit: Indigenous Corporate Training inc.

Trauma-Informed Care

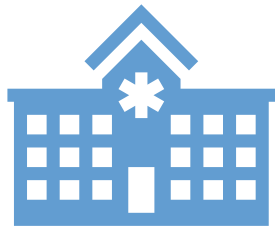
- Care that attends to a patient's past experiences of violence or trauma and the role it plays in their lives
- Not focused on treatment of psychological symptoms of trauma
- Provides service in a way that is responsive to the specific needs of those affected by trauma
 - ✓ Being respectful and non-judgmental
 - ✓ Creating a safe and welcoming environment
 - ✓ Being aware of how past trauma may influence behaviour
 - ✓ Avoiding actions that may trigger distress

Local Supports in Thunder Bay

- [Thunder Bay Indigenous Friendship Centre](#)
 - Provides cultural programming, meals, youth programs, Elders' support, and community connection for Indigenous people of all ages.
- [Dilico Anishinabek Family Care](#)
 - Offers child welfare, mental health, addictions services, and cultural healing programs grounded in Indigenous traditions.
- [Tikinagan Child and Family Services](#)
 - Focuses on child and family services, aiming to keep children connected to their families, communities, and culture.



Provincial Supports (Ontario)



Indigenous-led health and social services programs

Provide mental health, addictions care, and culturally safe healthcare services across Ontario.



Education and cultural initiatives

Support language revitalization, cultural teachings, and Indigenous curriculum integration in schools.

National Supports

- [Jordan's Principle](#)
 - Ensures First Nations children receive the services they need (health, education, social supports) without delays due to jurisdictional disputes.



Immigrants and Refugees

- From 2021 to 2022, there was an 8% increase in asylum refugees.
- Up to 500,000 people living in Canada are without status.
- Non-status individuals have limited or no access to public services.
- Sanctuary cities are committed to providing services to residents, regardless of immigration status.



Healthy Immigrant Effect

- The **healthy immigrant effect** is a health advantage that occurs during the first 2 years of living in Canada.
- Living in areas with a greater proportion of immigrant residents is a protective factor.
- Immigrants and refugees experience cultural, economic, geographical, and language barriers to accessing health care and social services.

Health Issues of Immigrants and Refugees

- Barriers to health care
 - Language barrier
 - Cultural differences
 - Low income
 - Difficulty understanding the healthcare system
 - Geographical barriers
- *Refugees may also experience trauma from forced migration
- Unique health needs
- Traditional medicine use



Homeless Populations

- **Homelessness:** not having a stable, safe, or permanent housing
- Range of living situations
 - Unsheltered
 - Emergency sheltered
 - Provisionally accommodated
 - At risk of homelessness



Why do you think homelessness happens?

Homeless Populations

- Causes of homelessness
 - Disinvestment in affordable housing
 - Structural shifts in the economy
 - Reduced spending on a range of social and health supports

Homeless Populations

Complex health issues

- Infectious and chronic diseases, cognitive issues, nutritional deficiencies, foot issues, mental health issues, substance use issues, and injuries

Barriers to care

- Lack of access
- Affordability issues (e.g., of medications and dental care)
- Lack of transportation
- Lack of knowledge about where to obtain care

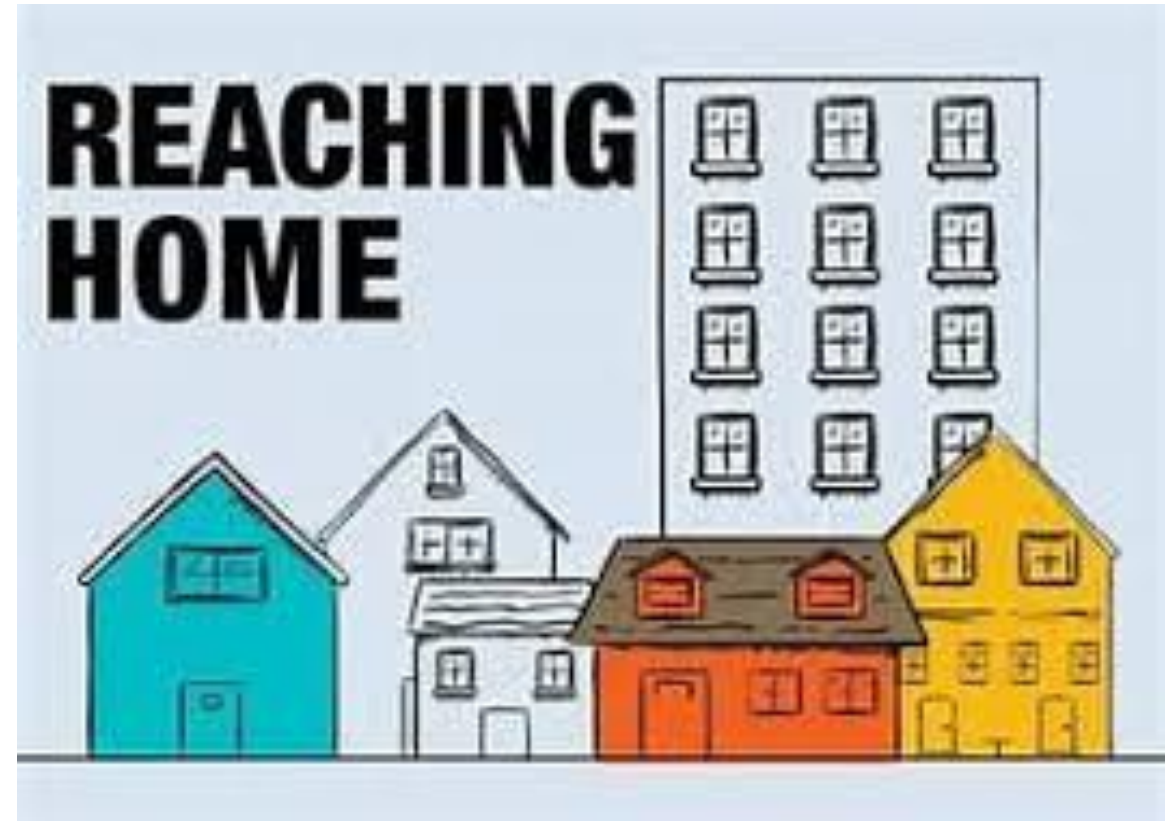
Strategies to Address Homelessness

Priority areas for support:

- Facilitating access to housing
- Mental health and addiction care
- Care coordination/case management
- Facilitating access to income

Federal Response to Homelessness

- [Reaching Home: Canada's Homelessness Strategy](#)
 - Aimed at preventing and reducing homelessness across Canada
 - Provides funding to urban, rural, remote, and Indigenous communities
 - Goal is to reduce chronic homelessness by 50% by 2028.



Gender- and Sexual-Diverse Populations

- Gender diversity
 - Goes beyond masculine and feminine
 - Includes two spirit, lesbian, gay, bisexual, transgender, queer, and intersex (2SLGBTQI+)
 - Gender and sexual diversity and minorities
 - Gender equity



Health Issues of Gender- Diverse Populations

- Inequalities
 - Higher suicide rates among 2SLGBTQI+ individuals
 - Homelessness among 2SLGBTQI+ youth is prevalent
 - Stigma, discrimination, and exclusion
 - Higher incidence of chronic conditions (e.g., asthma)

Trauma-Informed Care

- 2SLGBTQI+ people have a history of trauma, discrimination, stigma, and violence.
- Trauma-informed health assessment

Health Disparities

- Mental health concerns
 - Higher rates due to discrimination and social determinants of health
- Increased incidence of:
 - Substance use
 - For some used as a coping mechanism
 - Depression and anxiety
 - Self-harm and suicide

Mental Health: PSW Interventions

- Provide safe and affirming support:
 - Respect client's identity.
 - Use respectful communication.
 - Create safe and affirming environments.
 - Help provide access to supports.

Diverse Abilities Population

- **Diverse ability**
 - Variation in people's physical, intellectual, developmental, and cognitive abilities
- **Disability**
 - Defined as mental or physical condition that places a substantial or significant limit on an individual's ability to carry out some functions or activities
- 6.2 million Canadians live with some form of disability



Health Issues of Diverse Ability Populations

- Health inequities
 - Lack of access to health care
 - Treated dismissively
 - Experience long wait times
 - Lack of access to specialists
 - Challenges navigating the health care system

Response to Diverse Populations and Health

- PSWs can:
 - Work toward eliminating social inequities
 - Focus on providing culturally safe care
 - Conduct cultural nursing assessments

Next Class

- Required Readings:
 - Edelman & Kudzma (Ch. 8, 9)